CLINICAL STUDY PROTOCOL
Study ID: TX-2026-01 | Phase: III | Indication: Moderate-to-Severe Chronic Inflammatory Disease
Protocol Version: 1.0 (Baseline) | Effective Date: January 15, 2026
Sponsor: TrialSync Clinical Therapeutics, Inc.

1. STUDY SYNOPSIS AND OBJECTIVES

1.1 Primary Objective

The primary objective of this study is to evaluate the clinical efficacy of investigational agent TS-101 compared with placebo in achieving disease remission in adult participants with active inflammatory disease.

1.2 Secondary Objectives

- Assess the safety, tolerability, and immunogenicity profile of TS-101 across 24 weeks of exposure.
- Measure patient-reported quality of life and physical functioning using validated index instruments.

2. STUDY DESIGN AND POPULATION

This is a multicenter, randomized, double-blind, parallel-group Phase III investigation. Approximately 240 adult participants will be enrolled across 18 clinical sites in North America. Participants are randomized 1:1 to receive either TS-101 (200 mg subcutaneous biweekly) or matching placebo.

3. SCHEDULE OF ASSESSMENTS AND CLINIC VISITS
Participants attend scheduled clinic visits for screening, baseline randomization (Week 0), and regular monitoring visits every 4 weeks. Key assessments include physical examinations, laboratory safety panels, pharmacokinetics, and clinical disease activity indices.

section: endpoint
Primary endpoint assessment occurs at week 24.
The primary efficacy estimand is the proportion of participants achieving clinical remission (defined as a reduction in composite inflammatory score >= 50% from baseline) without rescue therapy. All primary statistical inferences will be conducted on the intent-to-treat (ITT) population.

4. SAFETY MONITORING AND DATA COMMITTEE
Adverse events and serious adverse events will be monitored continuously from informed consent through the end of safety follow-up. An independent Data Monitoring Committee (DMC) will review unblinded safety data at regular intervals.

5. HISTORICAL BENCHMARK COMPARATOR
Study assumptions and sample size calculations were informed by historical study CSR-2024-09, which evaluated early-generation biologic comparators under equivalent clinical trial conditions.